

TheraVoca

A different way to find the right therapist.

Free, anonymous therapist matching for Idaho.
Built around fit, not directory size.

Partner overview · 2026-05-18
Joshua Rosenthal, Founder
therapymatch@gmail.com

Executive summary

TheraVoca is a free service that helps people in Idaho find the right therapist in under ten minutes. A patient answers a short intake about what they're looking for; a scoring engine returns the three best-matched therapists, ranked by fit across thirteen different signals. The patient contacts the therapist directly. We are not in the room with them, and we never see clinical content.

Therapists pay \$45/month after a 30-day free trial. Patients pay nothing.

The thesis is simple. Today's options for finding a therapist are bad. Insurance directories are riddled with inactive providers. Psychology Today is a paid listing with no fit signal. Word-of-mouth is reliable but doesn't scale. People give up. A small, vetted, actually-available pool of therapists matched against an honest intake outperforms a giant directory of cold leads.

Where we are: the platform is built. Patient intake, matching, therapist signup, admin tools, billing, and the compliance posture are all live on staging. Public launch is weeks away pending two external blockers: SMS carrier approval and DNS migration.

At-a-glance

Market	Idaho, ~1.9M residents. ~1 in 5 adults needs a therapist; ~half of those who try to find on
Patient cost	Free.
Therapist cost	\$45/month, 30-day free trial. No setup fees, no per-referral fees.
Who's in the middle	Nobody. Patient contacts therapist directly via email, phone, or booking link.
HIPAA posture	Confirmed by attorney (2026-05-13): TheraVoca is NOT a Business Associate. We never
Status	Pre-launch. Platform shipped to staging. Public launch weeks away.

The problem

Finding a therapist who fits you is harder than finding a doctor, a dentist, or a lawyer. Three reasons:

1. Directories are stale.

Independent audits of insurance "find-a-provider" lists show roughly half of listed therapists are unreachable, not accepting new patients, or no longer practicing. Patients burn through five or ten calls before getting a callback. Many quit.

2. The listings carry no fit signal.

Sites like Psychology Today let therapists buy a placement, list their modalities, and write a bio. There's no scoring of whether this therapist actually matches what you're looking for. The patient has to read fifteen profiles and guess.

3. The intake doesn't go anywhere.

When a patient does fill out a contact form, it lands in an inbox the therapist might not check for days. By then the patient has either given up or moved on to the next name on the list. The information they shared is gone.

The net result: people who decide to start therapy spend weeks looking, get matched on availability rather than fit, and often quit within the first three sessions because the relationship doesn't click. The downstream cost (untreated anxiety, depression, substance use) is enormous and falls on primary care, ERs, employers, and families.

How it works -- the patient side

A patient who needs a therapist visits theravoca.com and answers four short sections of questions:

1. What's going on	Presenting issues (anxiety, depression, trauma, ADHD, relationship, etc). Urgency. Prior
2. What fits your life	In-person vs. telehealth, ZIP code, weekday vs. weekend, time of day.
3. How you pay	Insurance plan + member ID, or cash with a budget. Sliding-scale openness.
4. Optional deeper questions	Three open-text questions about how you describe what you're going through. Used for f

After the patient hits submit, we don't show results immediately. A 24-hour soft hold lets us fan the request out to a small set of qualifying therapists. Therapists who can actually take a new client write a short personal apply message that becomes part of the patient's view.

Twenty-four hours later, the patient sees their three top matches:

- A match-fit percentage (transparent: the patient can see the top reasons we matched, and the top reasons we didn't).
- The therapist's photo, credentials, modalities, years of experience.
- A short "Why we recommend" blurb that ties the therapist's actual practice to what the patient described.
- Concrete logistics: office address with a map link, accepted insurance, cash rate, sliding scale, free consult availability, languages spoken.
- Contact buttons that open the patient's email, phone, or calendar app -- no in-app messaging.

Important design decision: the patient takes the next step. We don't book sessions, hold payments, or sit between the patient and the therapist. This is what keeps us outside of HIPAA's Business Associate scope, and it's the simplest version of "matching done; rest is your relationship."

How it works -- the therapist side

Therapists sign up at theravoca.com/therapists/join and complete a nine-step profile. The profile is more detailed than typical directory listings because it powers the matching engine, not just the public page:

Basics	Name, credential type, email, public office phone, website, photo.
License	Idaho license number, expiration date, license photo upload (verified by admin).
Specialties	Primary (the matchmaker's first axis), secondary, general.
Format + modalities	Telehealth, in-person, or both. CBT, EMDR, DBT, IFS, etc.
Insurance + pricing	Insurance plans accepted, cash rate, sliding-scale availability, free 15-minute consult.
Style + bio	Style tags, a 200-300 word bio. AI-assisted draft option.
Deep-match signals	Six open-text + multiple-choice questions about how the therapist works (used to score fit)

Every signup is manually reviewed before going live. License is verified against the Idaho DOPL public registry. The therapist gets a 30-day free trial, then \$45/month via Stripe. They can pause matches at any time -- pausing during a busy stretch keeps them off the matching engine without canceling the subscription.

When a new patient comes in whose criteria match a therapist's profile, the therapist gets an email and SMS alert with a deep-link. They write a personal note that becomes the patient-facing apply blurb. Patients see the therapists who actually want to work with them, with a message in their own voice, instead of a directory of cold profiles.

The matching engine

Matching runs in two steps:

Step 1: Score every qualifying therapist against the patient

Each therapist gets a score from 0 to 95 based on thirteen weighted signals. The big ones:

Issues fit (35 pts)	Does the therapist specialize in the patient's primary concern?
Availability (20 pts)	Days and times the therapist works vs. when the patient is free.
Modality (15 pts)	Telehealth, in-person, or both -- does it match the patient's preference?
Deep-match resonance (15 pts)	Semantic similarity between the patient's open-text answers and the therapist's lived-experience
Urgency (10 pts)	Can the therapist take a new patient soon?
Payment alignment (13 pts total)	Insurance accepted + cash rate vs. budget + sliding scale.
Other signals (12 pts total)	Prior-therapy history, gender preference, language, experience-level preference, style over

Step 2: Re-rank by ability to engage

Once we have a qualifying pool, we don't just sort by the Step 1 score. We re-rank based on signals about whether each therapist is actually likely to write a personal apply message and respond when the patient reaches out:

- Speed: how fast did the therapist apply on their last few matches?
- Apply-message quality: was the last message generic, or specific to that patient?
- Commitment: has the therapist toggled themselves available, or do they sit unread?

The result: the patient sees three therapists who are both objectively a good fit AND have demonstrated they'll show up. The match-fit percentage and the "Why we matched" reasons are honest -- no marketing inflation -- and patients can see the gaps too ("Where you may not align: this therapist doesn't take Blue Cross"). Trust over hype.

Why Idaho first

We're starting in one state on purpose. Going state-by-state gives us four advantages:

1. One license verification regime

Idaho DOPL (Department of Occupational and Professional Licensing) maintains a public registry of every licensed therapist. We verify every signup against it. Multi-state expansion adds one regime per state.

2. One set of laws

Telehealth rules, scope-of-practice, and HIPAA-adjacent state privacy laws vary. Idaho is straightforward. Adding a state means a legal review pass, not an open-ended question.

3. A finite, mappable provider pool

There are roughly 1,800 licensed mental health providers in Idaho across LCSW, LMFT, LPC, psychologists, and psychiatrists. Big enough that patients always have options; small enough that we can build relationships with the providers ourselves.

4. A real underserved-area story

Idaho has the second-lowest per-capita mental health professional count in the country. Telehealth-equipped therapists in Boise can serve patients in Coeur d'Alene, Pocatello, Twin Falls, and rural counties that have zero in-person options. Matching makes that geography problem solvable.

The expansion plan, briefly: stabilize Idaho first, then add a second state where the same playbook works (license verification + a state-of-the-art directory + clear regulatory ground). Most likely Oregon, Washington, or Montana given proximity and overlap. Each new state takes weeks, not months, because the platform is already built.

Compliance and trust

TheraVoca handles sensitive intake content. We took it seriously from day one, and we have an attorney's written opinion confirming where we sit:

HIPAA posture

Per attorney Mason, dated May 13 2026: **TheraVoca is NOT a HIPAA Business Associate today.** We don't receive Protected Health Information on behalf of a covered entity. Patients self-disclose to us before any therapist relationship exists. What we collect is their own information voluntarily shared with a private service.

If we ever build features that move clinical data through us (session notes, clinical scheduling, billing on a provider's behalf), the BA analysis flips. We're explicit about staying clear of that line for the initial product.

Privacy practices

- Patients are anonymous in our system. We key requests by an internal ID and email only -- never name.
- No session recordings. Our analytics tool's session-replay feature is globally disabled.
- Error tracking has automatic redaction: emails, phone numbers, and OTP codes are stripped before any error event leaves the server.
- We never put patient information in URL parameters, cookies, or third-party tracking pixels.
- Email allowlist + override mode during pre-launch testing so test sends can't reach real patient inboxes.

Therapist verification

Every therapist signup goes through manual admin review before matches go live. License number and photo are verified against the Idaho DOPL registry. Soft-flag risk gates on patient intake escalate certain content (active crisis language, minors, sexualized content directed at the therapist) to admin for review before matching runs.

Current state

The platform is built. We're in pre-launch testing.

Shipped and live on staging

- Full patient intake (eight steps) with moderation gates.
- Full therapist signup (nine steps) with AI-assisted bio drafter.
- Matching engine with thirteen weighted signals and a two-step rank.
- 24-hour soft-hold gate.
- Therapist portal: profile editor, application queue, availability toggles.
- Admin dashboard: request review with score breakdown, provider directory with patient-view preview, moderation queue, email template editor, cron schedule manager, audit log.
- Stripe subscriptions with webhook signing.
- Cloudflare Turnstile bot protection.
- Sentry error tracking with PII auto-redaction.
- Idaho-only license validation, geographic matching, ZIP-based distance scoring.
- Video testimonials via Cloudflare Stream.
- Email templates with quiet-hours guard (no sends before 8 AM Idaho time).

External blockers before public launch

- **SMS carrier approval (Telnyx CTIA / A2P 10DLC):** applied; pending vetting. Email alerts work today; SMS is the second channel and not blocking.
- **DNS migration:** theravoca.com domain points to a placeholder. We have the Render service configured but haven't flipped the DNS yet.
- **Initial therapist roster:** ~30-50 real therapists is the soft-launch minimum so patients see meaningful options. Currently a mix of test data and a handful of real early signups.

What we're looking for

Depending on the partnership, useful introductions land in one of four buckets:

1. Therapists who'd be willing early adopters

The first thirty to fifty therapists matter more than the next five hundred. We want providers who'll log in, write real apply messages, and give us feedback on what's missing. The 30-day free trial covers their first month at zero cost.

2. Idaho-based referral sources

Primary care practices, school counseling offices, university wellness centers, employer assistance programs. Anyone who currently hands patients a stale list and hopes for the best. TheraVoca is a better referral destination -- patients get matched in ten minutes instead of fifteen calls.

3. Strategic / institutional partners

Insurers, hospital systems, telehealth platforms that need a fit-based referral layer. Our patient handoff model keeps us out of the clinical relationship, which means we can plug into a partner's existing intake or provider directory without triggering BA agreements or data-flow restructuring.

4. Advisors with operating experience

Founders or operators with experience in two-sided marketplaces, regulated healthcare, or Idaho-specific GTM. We have working product and a defensible scoring engine; we'd benefit from people who've grown a similar shape of business before.

Contact

Joshua Rosenthal, Founder

therapymatch@gmail.com

Demo site: theravoca-production.onrender.com (staging; ask for the password)

Generated 2026-05-18. This document is a snapshot; the platform ships changes daily.